Clinical Practice Guideline: Primary Care Management of Chronic Kidney Disease (Abridged)

Guideline ID: DEMO-CKD-2025-A **Version:** 1.0 (demonstration abridgement) **Effective Date:** April 2025 **Issuing Body:** Adapted for demonstration from the VA/DoD Clinical Practice Guideline for the Primary Care Management of Chronic Kidney Disease, Version 5.0 (April 2025)

1. Scope and Purpose

1.1 Clinical Condition

This guideline covers kidney-protective therapy, laboratory monitoring, and nephrology consultation for adults with chronic kidney disease not on dialysis. It is a demonstration abridgement that keeps a small number of the source guideline's recommendations, with their original numbers, and omits the rest.

1.2 Target Population

Adults with chronic kidney disease, defined by an estimated glomerular filtration rate (eGFR) below 60 mL/min/1.73 m2 or a urine albumin-to-creatinine ratio (UACR) above 30 mg/g, managed in primary care.

1.3 Exclusions

Dialysis, kidney transplantation, polycystic kidney disease, and contrast-associated kidney injury are outside the scope of this abridgement.

2. Background

Kidney protection in the source guideline rests on renin-angiotensin system inhibition for patients with hypertension and albuminuria, the addition of a sodium-glucose cotransporter-2 inhibitor for defined subgroups, and timely nephrology consultation for patients with advanced or rapidly progressing disease.

3. Recommendations

3.1 Kidney-Protective Therapy (source recommendations 12, 14, 15)

For patients with hypertension and albuminuria (UACR above 30 mg/g), either an ACE inhibitor or an angiotensin II receptor blocker is recommended to slow progression (Strong for). Patients with an eGFR below 30 mL/min/1.73 m2 who are already on one of these agents continue therapy unless there is intolerance or an adverse event (Weak for). The addition of a sodium-glucose cotransporter-2 inhibitor to a maximally tolerated ACE inhibitor or angiotensin II receptor blocker is recommended for patients who have one or more of type 2 diabetes, albuminuria with UACR above 200 mg/g, or heart failure (Strong for).

Decision Table 1: Protective Therapy

Has Hypertension	UACR (mg/g)	Has Type 2 Diabetes	Has Heart Failure	Indicated Therapy
Yes	> 200	Any	Any	ACE inhibitor or ARB, plus SGLT2 inhibitor
Yes	30 to 200	Yes	Any	ACE inhibitor or ARB, plus SGLT2 inhibitor
Yes	30 to 200	No	Yes	ACE inhibitor or ARB, plus SGLT2 inhibitor
Yes	30 to 200	No	No	ACE inhibitor or ARB
Any	<= 30	Yes	Any	SGLT2 inhibitor
Any	<= 30	No	Yes	SGLT2 inhibitor
No	<= 30	No	No	No therapy indicated by this table

Key principles:

1 The SGLT2 inhibitor is added to a maximally tolerated ACE inhibitor or angiotensin II receptor blocker and continued until the start of dialysis.

1 An ACE inhibitor and an angiotensin II receptor blocker are never combined.

3.2 Nephrology Consultation (source algorithm sidebar 8; recommendation 9)

Nephrology consultation is indicated for an eGFR below 30 mL/min/1.73 m2; for a rapid decline of eGFR of more than 5 mL/min/1.73 m2 per year; for a patient without diabetes with confirmed heavy albuminuria (UACR above 300 mg/g); and for a patient with diabetes with persistent albuminuria above 1000 mg/g despite renin-angiotensin system and SGLT2 inhibition.

Decision Table 2: Nephrology Consultation

eGFR (mL/min/1.73 m2)	UACR (mg/g)	Has Diabetes	Consultation Indicated
< 30	Any	Any	Yes
>= 30	> 300	No	Yes
>= 30	> 1000	Yes	Yes
>= 30	<= 300	No	No
>= 30	<= 1000	Yes	No

Key principles:

1 A rapid decline of eGFR of more than 5 mL/min/1.73 m2 per year is also an indication for consultation; it is assessed by the clinician from the trend of results and is not part of the table.

3.3 Laboratory Monitoring (source algorithm box 5; laboratory monitoring table)

Serum creatinine, eGFR, and a spot UACR are obtained at least annually. Serum potassium is checked within 2 to 4 weeks of initiation or a dose increase of an ACE inhibitor or angiotensin II receptor blocker.

3.4 Annual Kidney Monitoring Process

Every 12 months the care team checks the record for an eGFR result and a UACR result within the preceding 12 months. For each result that is missing, the care team requests the laboratory order and notifies the responsible clinician. When both results are present, the care team re-evaluates Table 2 and, if consultation is indicated, notifies the responsible clinician and requests a nephrology referral visit.

3.5 Post-Initiation Potassium Check Process

When an ACE inhibitor or angiotensin II receptor blocker is started or its dose is increased, the care team requests a serum potassium result for 2 to 4 weeks after the change. If the result is not available by the end of that window, the care team notifies the responsible clinician.

Key principles:

1 Decision Tables 1 and 2 are the only decision logic in this guideline. The processes in 3.4 and 3.5 use record queries, laboratory requests, visit requests, and notifications; they do not define additional decisions.

3.6 Statin Therapy (source recommendation 19)

For patients with chronic kidney disease not on dialysis, initiation of a statin is recommended to reduce major adverse cardiovascular events and mortality (Strong for).

4. Evidence Summary

Recommendation	Source number	Strength
ACE inhibitor or ARB with hypertension and albuminuria	12	Strong for
Continue ACE inhibitor or ARB at eGFR below 30	14	Weak for
Add SGLT2 inhibitor in defined subgroups	15	Strong for
Nephrology referral with sufficient preparation time	9	Weak for
Statin initiation in CKD not on dialysis	19	Strong for

5. References

1 Department of Veterans Affairs, Department of Defense. VA/DoD Clinical Practice Guideline for the Primary Care Management of Chronic Kidney Disease, Version 5.0. April 2025.